Letrozole · daily hormone therapy

Take letrozole once daily continuously. The prescriber confirms whether the aim is before surgery, after surgery, extended adjuvant treatment or advanced-disease control.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Read the row for your treatment setting. Columns show order, not equal lengths of time. Agree the due window, booking team, result reviewer and whom to contact if a check is missing.

Check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Bloods / symptom review	full blood count (FBC), renal and liver profile	Review tolerability and medicines	Further tests when clinically indicated	Review symptoms, adherence and results	Follow unresolved abnormalities
electrocardiogram (ECG) / echo	No routine ECG / echo for this medicine alone	Investigate new cardiac symptoms	Further cardiac tests by risk / symptoms	Companion-drug cardiac rules remain separate	Follow abnormalities when indicated
Bone density / dual-energy X-ray absorptiometry (bone density) (DXA)	Adjuvant aromatase inhibitor (AI): baseline DXA unless already on adjuvant bisphosphonates	Review previous DXA and fracture risk	No cycle-based DXA appointments	Lower-risk AI: reassess in 1–2 years; individualise	Continue bone care according to risk
Early-breast surveillance	Confirm early / curative setting	Set first follow-up mammogram date	Assess new symptoms promptly	Annual mammography of remaining breast tissue	No routine computed tomography (CT) / markers if asymptomatic
Advanced-disease response	Document disease sites / baseline assessment	New symptoms can bring assessment forward	Generally every 2–4 months; individualise	Adapt interval and imaging to disease / benefit	Markers supplement, never replace, assessment

Check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Before-surgery pathway only	Confirm hormone receptors, HER2 and the chemotherapy / surgery plan	Agree clinical response / surgery reviews	Assess response during 4–8 months	Inadequate response: surgery / alternative review	Proceed to agreed operation / next treatment

THE WHOLE TREATMENT JOURNEY

Letrozole · daily hormone therapy: treatment and checks

Postmenopausal hormone-sensitive breast cancer; setting-specific pathways

Take letrozole once daily continuously. The prescriber confirms whether the aim is before surgery, after surgery, extended adjuvant treatment or advanced-disease control.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Treatment plan NCCP 00371	Confirm indication and prescription	Start the agreed daily / injection schedule	Continue unless prescriber changes the plan	Review duration, benefit and tolerability	Agree continuation, switch or completion
Bloods / symptom review NCCP 00371	FBC, renal and liver profile	Review tolerability and medicines	Further tests when clinically indicated	Review symptoms, adherence and results	Follow unresolved abnormalities
ECG / echo NCCP 00371	No routine ECG / echo for this medicine alone	Investigate new cardiac symptoms	Further cardiac tests by risk / symptoms	Companion-drug cardiac rules remain separate	Follow abnormalities when indicated
Bone density / DXA NCCP 00371 · AIBL2025	Adjuvant AI: baseline DXA unless already on adjuvant bisphosphonates	Review previous DXA and fracture risk	No cycle-based DXA appointments	Lower-risk AI: reassess in 1–2 years; individualise	Continue bone care according to risk
Early-breast surveillance NCCP 00371	Confirm early / curative setting	Set first follow-up mammogram date	Assess new symptoms promptly	Annual mammography of remaining breast tissue	No routine CT / markers if asymptomatic

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Advanced-disease response NCCP 00371	Document disease sites / baseline assessment	New symptoms can bring assessment forward	Generally every 2–4 months; individualise	Adapt interval and imaging to disease / benefit	Markers supplement, never replace, assessment
Before-surgery pathway only NCCP 00371	Confirm hormone receptors, HER2 and the chemotherapy / surgery plan	Agree clinical response / surgery reviews	Assess response during 4–8 months	Inadequate response: surgery / alternative review	Proceed to agreed operation / next treatment
Early: radiotherapy ESMO_EBC2024	Review surgery / pathology	Plan radiotherapy if indicated	Coordinate endocrine timing	Follow the early-breast plan	Review late effects when relevant

What each check is for

1 Confirm the plan

Before starting or changing treatment

Sources: NCCP 00371

2 Checks and symptoms

At the agreed review points

Sources: NCCP 00371

3 Review ongoing benefit

At setting-specific decision points

Sources: NCCP 00371

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.20 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending